INFORMATION TO KEEP WITH YOU

My complete treatment handbook

procarbazine, lomustine and vincristine (PCV) chemotherapy · 56-day cycle

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
56 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00658
Version 2

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 56 days. Six cycles are planned, with review before treatment.

Treatment stages

01 **PCV · 56-day cycle**
Three medicines on different days

02 **Recovery and the next-cycle decision**
Delayed blood-count effects

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Use your written emergency plan even on days without chemotherapy. Fever, shivering or suddenly feeling unwell needs urgent advice. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow any existing seizure rescue plan. Promptly report worsening headache, repeated vomiting, cough, breathlessness, unusual bruising, yellow skin or eyes, or inability to drink. A held chemotherapy dose must not be restarted without instructions.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

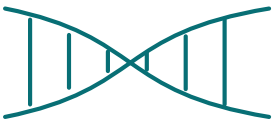
How your treatment works

This eight-week PCV regimen combines procarbazine, lomustine and vincristine after radiotherapy for grade II glioma. It aims to improve disease control. NCCP describes this indication as unlicensed; the treating team explains the evidence, alternatives and local consent process.

ORAL CHEMOTHERAPY CAPSULE

Lomustine

CCNU



Damages deoxyribonucleic acid (the cell's genetic material) (DNA)

Lomustine damages genetic material so cells cannot divide normally. It can affect cancer cells and healthy bone-marrow cells. Blood counts may fall several weeks after the single dose, so a day without capsules is not a day without risk.

One scheduled dose → DNA damaged → Delayed blood-count effects possible

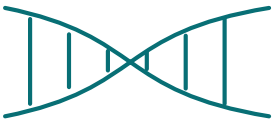
What to watch for

Take only the prescribed Day 1 dose. Do not take it daily or repeat it without oncology instructions. Report fever, bleeding or new breathing symptoms.

ORAL CHEMOTHERAPY CAPSULE

Procarbazine

Procarbazine capsules



Interferes with DNA and cell growth

Procarbazine is converted in the body into substances that interfere with DNA and cell growth. Healthy blood-forming and digestive tissues can also be affected. It has important interactions with some medicines, alcohol and certain foods, so a pharmacy review and written instructions are essential.

Capsules on specified days → DNA processes disrupted → Tumour and healthy cells affected

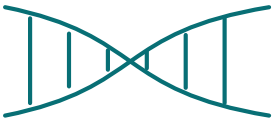
What to watch for

Follow the exact Days 8–21 calendar and the pharmacist's medicine, food and alcohol instructions. Report fever, rash or persistent vomiting.

CHEMOTHERAPY GIVEN INTO A VEIN

Vincristine

Vincristine infusion



Disrupts the machinery used for cell division

Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Infusion into a vein → Cell division disrupted → Nerves and bowel may be affected

What to watch for

Report tingling, weakness, walking difficulty or constipation before the next dose. Tell staff immediately if the infusion site stings, burns or swells.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment and its purpose

Use the regimen and phase agreed with your team.

What each part does

Lomustine damages genetic material so cells cannot divide normally. It can affect cancer cells and healthy bone-marrow cells. Blood counts may fall several weeks after the single dose, so a day without capsules is not a day without risk. Procarbazine is converted in the body into substances that interfere with DNA and cell growth. Healthy blood-forming and digestive tissues can also be affected. It has important interactions with some medicines, alcohol and certain foods, so a pharmacy review and written instructions are essential. Vincristine interferes with structures that cells use when dividing. It can also affect nerves controlling sensation, strength and bowel movement. This explains why tingling, weakness and constipation need early review even if blood tests are satisfactory.

Schedule and planned duration

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 56 days. Six cycles are planned, with review before treatment.

Evidence and uncertainty

Adding PCV improved long-term survival in the study population. This evidence does not automatically apply to every grade II glioma or to recurrent high-grade disease. The study used historical pathology classification; the current integrated diagnosis, risks, expected tolerance and patient priorities guide individual decisions.

Understanding your particular glioma

Your diagnosis includes the tumour's cells, molecular findings and behaviour, not just the word brain cancer.

The name and location

A glioma is a tumour arising in the brain or spinal cord. Astrocytoma, oligodendroglioma and glioblastoma are different types. Cancer that travels to the brain from another organ is a brain metastasis and follows a different pathway.

Grade is not the same as stage

Glioma reports combine the tumour type, grade and molecular findings. Grade is not a measure of spread around the body: a grade 4 brain tumour does not mean body-wide metastases. Ask your specialist to explain older terms in your records.

isocitrate dehydrogenase (IDH) and 1p/19q

Changes in the IDH genes and loss of chromosome regions 1p and 19q help classify certain gliomas. The pattern of results matters. These are usually tumour-sample tests; they do not by themselves show that a change runs in the family.

O6-methylguanine-DNA methyltransferase (MGMT) promoter methylation

The MGMT gene makes a protein that repairs a particular type of DNA damage. Methylation in its control region can reduce that repair and inform discussion of temozolomide benefit. This is a tumour-tissue result, not a blood marker or a personal survival percentage.

A useful diagnosis record

Keep the exact pathology diagnosis, sample date, grade and available molecular results together. Label a result as pending or not tested when appropriate. Ask whether further pathology review would affect the plan.

Your overall treatment plan and choices

Treatment should account for the tumour and the life you want to lead.

Understand the sequence

Place biopsy or surgery, radiotherapy, medicines and follow-up on one plan. Concurrent means treatments overlap; adjuvant means treatment follows the main treatment. A new phase can require different doses, tests and support medicines.

Agree what treatment is trying to achieve

Ask what treatment can reasonably achieve for your tumour and which effects matter most to you. Discuss benefits, burdens, alternatives and the reasons for a pause or stop.

Put evidence in context

A study result applies to a particular group, diagnosis and schedule. Older studies may use different tumour names. Ask which evidence fits your plan; an average cannot forecast your response or lifespan.

Discuss options available to you

Ask about suitable trials, another specialist opinion and other treatment or symptom-care options. Your multidisciplinary team explains what is appropriate and available for you.

Make room for your priorities

Bring up thinking, independence, family time, work and the side effects you most want to avoid. Ask for time, written notes or a supporter if that helps you weigh the choices.

Radiotherapy followed by PCV in selected grade II glioma

RTOG 9802 compared radiotherapy alone with radiotherapy followed by six eight-week cycles of PCV.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment days → recovery and tests → review

Treatment stages

01 **PCV · 56-day cycle**
Three medicines on different days

02 **Recovery and the next-cycle decision**
Delayed blood-count effects

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE MEDICINES ON DIFFERENT DAYS

PCV · 56-day cycle

D1 lomustine | **D8–21 procarbazine** | **D8 + D29 vincristine**

Lomustine once on Day 1; procarbazine daily on Days 8–21; vincristine into a vein on Days 8 and 29. The cycle lasts 56 days. Six cycles are planned, with review before treatment.

DELAYED BLOOD-COUNT EFFECTS

Recovery and the next-cycle decision

Review before Day 57

The next cycle can be delayed or changed according to blood counts, organ tests, nerves, bowel symptoms and the treatment goal. A calendar or card stamp is not permission to restart.

For adjuvant treatment the source plans PCV to begin within four weeks after radiotherapy. The two PCV cycle lengths are separate regimens; do not interchange their calendars.

BASELINE → TREATMENT → FOLLOW-UP

Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed magnetic resonance imaging (MRI) and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00658 v2 · current regimen · Lomustine medac current product information · January 2026

THE WHOLE TREATMENT JOURNEY

PCV chemotherapy · 56-day cycle: treatment and checks

NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits / recovery	Next-cycle / response review	After a pause or stop
Blood count / kidneys / liver NCCP 00658	full blood count (FBC), renal/liver profile, glucose; baseline lung function	FBC, renal and liver tests before each treatment	Lomustine SmPC: blood count at least weekly for at least 6 weeks after each dose	Review the lowest counts and recovery before another cycle	Follow persistent blood, lung, kidney or liver effects
Lungs / bowel / peripheral nerves NCCP 00658	Baseline lung function; record bowel and nerve symptoms	Review before treatment; repeat lung testing with prolonged therapy or symptoms	Call for cough, breathlessness, severe constipation or new weakness	Check whether each PCV component remains suitable	Continue review of unresolved effects
Neurology / medicines / function NCCP 00658	Record neurological symptoms, daily function and current medicines	Review seizures, headache, cognition, mood, walking and any steroid plan	Report changes early; do not wait for the next MRI	Review treatment tolerance, medication support and patient goals	Agree follow-up and rehabilitation or symptom support
MRI / diagnosis NCCP 00658	Confirm integrated diagnosis, molecular results and baseline imaging	MRI timing according to the neuro-oncology plan	Keep the booking and result-review contact in your passport	Interpret scan with symptoms, treatment effects and clinical review	Agree the next imaging and clinical review

What each check is for

1 PCV · 56-day cycle

Three medicines on different days

This eight-week PCV regimen combines procarbazine, lomustine and vincristine after radiotherapy for grade II glioma. It aims to improve disease control. NCCP describes this indication as unlicensed; the treating team explains the evidence, alternatives and local consent process.

Ask / act: Confirm the current phase, prescription, blood tests and support medicines.

2

Recovery and the next-cycle decision

Delayed blood-count effects

This eight-week PCV regimen combines procarbazine, lomustine and vincristine after radiotherapy for grade II glioma. It aims to improve disease control. NCCP describes this indication as unlicensed; the treating team explains the evidence, alternatives and local consent process.

Ask / **act:** Confirm the current phase, prescription, blood tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking the treatment

Lomustine is taken once on Day 1 only; procarbazine is taken daily on Days 8–21; vincristine is given into a vein on Days 8 and 29. Follow the pharmacy label for capsule number, food and timing. Do not open or crush capsules. Lomustine is not a daily medicine. Vincristine must only be given intravenously by trained staff.

STEP 2

Missed or vomited medicine

If a capsule dose is missed or vomited, contact the oncology pharmacy for advice. Do not take extra capsules, repeat the single lomustine dose, or extend procarbazine days without instructions.

STEP 3

Medicine and supplement review

Bring an up-to-date list of all medicines, including steroids, seizure medicines, anticoagulants, non-prescription treatments, vitamins and herbs. Ask before starting something new. Keep a record of all dose changes.

STEP 4

Procarbazine interactions

Avoid alcohol during treatment. Ask for a written food list and medicine interaction check, including the period after procarbazine when precautions still apply. Do not assume an over-the-counter cold remedy, antidepressant, pain medicine or herbal product is safe without pharmacy review.

STEP 5

Storage and handling

Keep capsules in their original labelled packs, away from children. Do not open, crush or chew. If swallowing or medicine organisation is difficult, contact the team before the next dose and agree safe help. Return unused chemotherapy to the pharmacy.

STEP 6

Pregnancy, breastfeeding and fertility

Treatment can harm a pregnancy and may affect fertility. Discuss fertility preservation, contraception for patients and partners, and the required period after the final dose before starting. Do not breastfeed during treatment. Ask the team for written, medicine-specific advice.

STEP 7

Appointments and transport

Before starting: full blood count, kidney and liver tests, glucose and baseline lung-function assessment. NCCP requires blood count, kidney and liver tests before each treatment. Low counts can be delayed and cumulative after lomustine, so keep any additional blood tests the team arranges. Repeat lung-function assessment during prolonged therapy or when clinically indicated. MRI timing follows the individual glioma plan. Current lomustine product information additionally advises weekly blood counts for at least six weeks after each dose; the team should arrange these alongside the NCCP pre-treatment checks.

STEP 8

Help with the daily plan

Agree who can help with medicines, travel and appointments if memory, sight, fatigue or hand strength makes them difficult. Record your preferences and consent for sharing information.

Taking treatment safely

The exact prescription and calendar matter.

Missed or vomited medicine

Medicine and supplement review

Tests, urgent symptoms and follow-up

Keep planned tests and know how to get help.

Support medicines

Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan. Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice. Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them. Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.

A delay is a treatment decision

Blood counts, organ tests or symptoms may mean a dose is held, reduced or stopped. This is a safety decision, not a reason to take extra capsules later. Ask which medicines change, which support medicines continue and when the next assessment is due.

After the final dose

Agree follow-up blood tests, symptom review, MRI timing and who to contact. Continue to report new problems; effects can outlast chemotherapy. Keep your medicine list and written plan current.

Tests and scans: what each one tells you

Safety blood tests, tumour tissue tests and MRI answer different questions.

Safety blood tests

A full blood count measures red cells, white cells and platelets. Kidney and liver tests assess other aspects of treatment safety. Follow the blood-test schedule even when you feel well or the scan is reassuring.

MRI and neurological review

MRI gives detailed pictures of the brain. The team also checks symptoms, movement, speech, thinking and daily function. Before scanning, tell staff about implants, metal, kidney problems or previous contrast reactions, and ask for help if the scan worries you.

Changes on a scan need interpretation

Treatment-related changes can resemble tumour growth on MRI; this is called pseudoprogression. The specialist interprets the scan with symptoms and may need another scan or further assessment. Report worsening symptoms promptly rather than assuming a scan change is harmless.

An individual follow-up timetable

Record the next MRI and the contact who will explain it. Your diagnosis, treatment phase and previous results determine the interval. Other scans or blood markers are ordered for a specific reason, not automatically before every cycle.

Keep results with decisions

Mark tests as booked, done and reviewed so the next action is clear. Contact the named service about a missing result. Do not change chemotherapy or steroids in response to a report without instructions.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 56: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Lomustine: single dose	 **Day 1** Take once on Day 1 only, not daily.
Procarbazine daily	 **Days 8–21** Use the exact 14-day calendar and pharmacy interaction instructions.
Vincristine infusion	 **Day 8** intravenous (IV) treatment after review.
Second vincristine infusion	 **Day 29** Review bowel and nerve symptoms before treatment.
Recovery and safety checks	 **Days 30–56** Counts can be low after capsule days finish.
Symptoms throughout the cycle	 **Days 1–56** Call for fever, bleeding, breathlessness or severe bowel symptoms.

Lomustine is a single dose: Taking lomustine daily by mistake can cause serious harm. Keep the Day 1 instruction visible and ask the pharmacist to check your understanding.

Delayed marrow effects: Lomustine can cause delayed and cumulative suppression of blood counts. Rest days still need infection and bleeding awareness, and some cycles need delay or adjustment.

Fourteen procarbazine days: Use Days 8–21 inclusive from the treatment table. Do not add Day 22 from conflicting wording elsewhere; ask pharmacy if a supplied calendar differs.

Weekly blood counts after lomustine: The current product information calls for blood counts at least weekly for at least six weeks after a dose. Keep the arranged checks even when you feel well and on days without chemotherapy.

Why these bands stay broad: The cycle view shows relative treatment days, not a prediction of when symptoms will peak. Serious symptoms can occur at any time, including rest days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00658 v2 · current regimen ↗

There are no guaranteed "safe days". Fever, shivering or suddenly feeling unwell needs urgent advice, including on treatment rest days. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. Follow the emergency instructions in any established seizure rescue plan.

Between-treatment notes

Day 1–7: One lomustine dose on Day 1

Take only the labelled Day 1 dose. No procarbazine is scheduled until Day 8. Check the capsule supply and anti-sickness plan; do not repeat lomustine.

Day 8–21: Daily procarbazine and first vincristine

Take procarbazine each day from Day 8 through Day 21. Attend the Day 8 vincristine infusion and required tests. Follow the bowel, alcohol, food and medicine-interaction plan.

Day 22–29: Procarbazine finished; Day 29 infusion

There are no planned procarbazine doses after Day 21. Vincristine is scheduled again on Day 29 after assessment. Report numbness, weakness or constipation before the infusion.

Day 30–56: Recovery before the next cycle

No scheduled PCV medicines in this period. Blood-count effects can still develop or persist. Attend tests and restart only after team review.

Support around the medicines

1 **Food, alcohol and medicine review**

Brain symptoms, medicines and when to call

Symptoms may reflect the tumour, swelling, treatment, another medicine or a separate illness.

Know your usual symptoms

Before treatment, note your usual walking, strength, speech, sight, memory, sleep and seizure pattern. Record new changes and their timing. A trusted person can help if you want them involved.

Steroids need a separate written plan

Steroids may reduce swelling around a brain tumour. If needed, ask for a written dose, review date and reduction plan; do not stop suddenly or adjust the dose yourself. Report infection symptoms, marked thirst, weakness or troublesome sleep or mood changes.

Seizure medicines and rescue instructions

Take prescribed seizure medicine on its own schedule. Ask what to do after a missed or vomited dose and how to manage a breakthrough seizure. Do not switch or stop it yourself. Anyone giving a rescue medicine needs the personal instructions and training.

Safety at home and outside

Discuss driving, work, bathing, swimming, heights and machinery with your team, taking account of symptoms and seizures. Individual medical and licensing advice is needed. Occupational therapy can help make daily routines safer.

Living well, rehabilitation and support

Practical and symptom support can begin alongside anticancer treatment.

Thinking, communication and daily tasks

Tell the team about changes in memory, language, attention, sight or planning. Rehabilitation can involve physiotherapy, occupational therapy, speech and language therapy or psychology. Ask for practical help with routines and medicines.

Food, nausea, bowels and energy

Use your prescribed nausea and bowel plan and report problems early. Ask for dietitian help if intake or weight falls. Adapt movement to your balance and strength, and ask the team to assess fatigue that disrupts daily life.

Emotional support and carers

Tell the team about changes in mood, behaviour or coping. Patients and carers may each need psychological or practical support. Agree who can receive information and ask for help before caring becomes overwhelming.

Work, transport and finances

Plan travel, time away from work, caring duties and help with appointments. A nurse or social worker can identify practical support; agree who can help with tablets if needed.

Supportive care and future wishes

Supportive and palliative care can help manage symptoms while treatment continues. When you are ready, discuss future care preferences and whom you want involved. Ask your team to help document these wishes.

Information that works on paper

Keep the latest paper schedule, medicine list, results plan and emergency contacts together. The treatment card helps others identify your regimen. It supports, but does not replace, the prescription and emergency instructions.

Questions worth asking

What is my exact diagnosis, grade and relevant molecular result?

Ask for the integrated pathology wording and an explanation of IDH, 1p/19q and MGMT where relevant. The diagnosis and treatment history guide how this regimen applies to you.

Can you check the Day 1 lomustine and Days 8–21 procarbazine instructions?

Ask the pharmacist to mark each medicine on the calendar. Lomustine is a single Day 1 dose; procarbazine is a different medicine taken over fourteen days. Confirm food, alcohol, interaction and bowel instructions.

Who helps with seizures, steroids, memory or daily tasks?

Ask for a named contact, written seizure and steroid plans if prescribed, and rehabilitation or practical support. Agree who can help you organise treatment.

When will we review the benefit and my priorities?

Ask for the next clinical and MRI review, how treatment effects will be distinguished from growth, and what would lead to continuing, changing or stopping treatment.

Try explaining the plan back: Can you show the days for each medicine, the current phase, your next blood tests and whom to call for fever, vomiting or new neurological symptoms?

PCV · 56 days NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy. Use the plan matched to your diagnosis and phase; do not borrow another glioma schedule.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: lomustine

Active medicines: Lomustine

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	Before Day 1 lomustine, then the prescribed delayed-emesis cover NCCP V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.

Additional phase-specific support

- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Days 8-21: procarbazine; vincristine on Day 8

Active medicines: Procarbazine + Vincristine on Day 8

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Oral 5-HT3 antagonist - select ONE	Ondansetron 8-16 mg total daily OR granisetron 1-2 mg total daily; prescriber selects and records agent/ dose	Daily during procarbazine Days 8-21; confirm the prescribed stop date after this multiday phase NCCP V6 Table 5, p8. Start at least 30 minutes before oral systemic anticancer therapy (SACT) (p4). Confirm stop date, delayed-emesis cover and rescue plan; review QT risk, liver function, constipation and interactions.

Medicine	Dose / route	When
Constipation prevention	Complete the locally prescribed laxative and dose	With vincristine exposure; review bowel function before each infusion NCCP PCV supportive care recommends prophylaxis. Severe pain, vomiting or inability to pass stool/wind requires urgent assessment, not automatic extra laxatives.

Additional phase-specific support

- NCCP treatment table specifies procarbazine Days 8-21; the supportive-care paragraph says Days 8-22. Do not add chemotherapy on Day 22. Pharmacy confirms the antiemetic stop date.
- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last SACT day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 29: vincristine only

Active medicines: Vincristine

Emetogenic risk: Minimal

Medicine	Dose / route	When
Constipation prevention	Complete the locally prescribed laxative and dose	With vincristine exposure; review bowel function before each infusion NCCP PCV supportive care recommends prophylaxis. Severe pain, vomiting or inability to pass stool/wind requires urgent assessment, not automatic extra laxatives.

Additional phase-specific support

- No routine antiemetic prophylaxis is required for vincristine alone; prior symptoms or other medicines may require an individual rescue plan.

- Lomustine and procarbazine have moderate-to-high oral emetic potential. Vincristine alone has minimal emetic risk. The team supplies a day-specific anti-sickness plan, including the procarbazine days, and a rescue plan.
- Vincristine can slow the bowel through its effect on nerves. Preventive laxatives are recommended in NCCP; follow the prescribed plan and report constipation early. Severe abdominal pain, vomiting or inability to pass stool or wind needs urgent advice.
- Procarbazine has important interactions. Avoid alcohol during treatment and follow the pharmacist's written food restrictions and advice about when restrictions end. Check all prescription, over-the-counter, herbal and supplement products before taking them.
- Keep prescribed steroids and seizure medicines on a separate plan from chemotherapy. Before vincristine, report new weakness, tingling, walking difficulty or constipation. Check new symptom medicines with the team.
- These are prescription choices for clinician/pharmacist completion, not an issued prescription. General NCCP multiday guidance includes cover after the last systemic anticancer therapy (SACT) day; record the individual duration. Review pre-existing steroid and seizure treatment rather than duplicating it.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Brain / energy

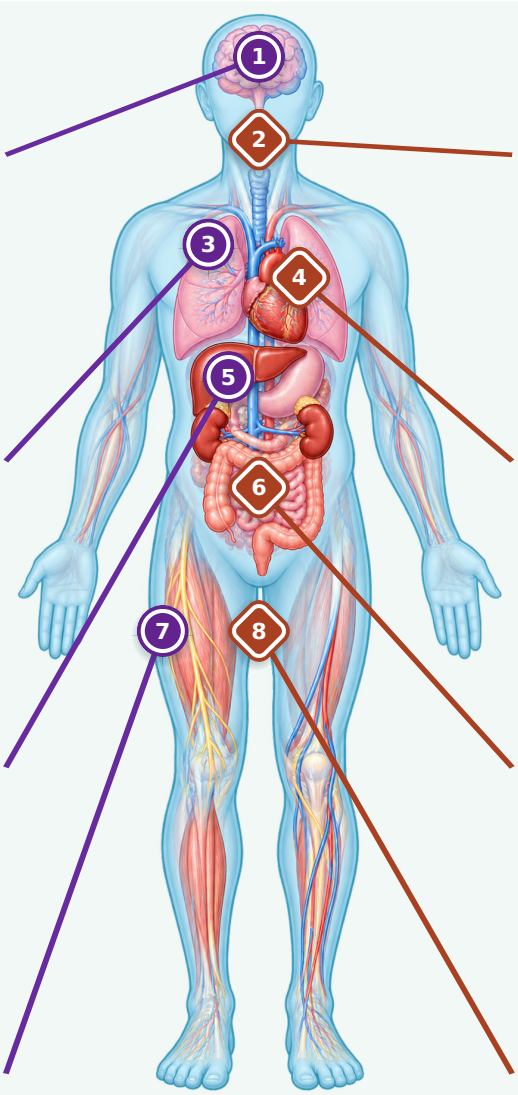
Glioma / PCV

Fatigue or neurological changes need review. New seizures, sudden weakness or speech difficulty need emergency assessment.

2 Mouth / eating

PCV

Mouth soreness or appetite loss can affect intake. Keep mouth care and ask for help with pain, ulcers or difficulty eating.



3 Blood / infection

Lomustine / procarbazine

Counts may fall weeks after lomustine and worsen over cycles. Fever, sudden illness, bruising or bleeding needs urgent advice.

4 Lungs

Lomustine

New cough or breathlessness needs assessment. Lung injury can develop with treatment; keep baseline and any repeat lung tests.

5 Liver / kidneys

PCV

Attend organ-function blood tests. Report yellow skin or eyes, dark urine, reduced urine or dehydration rather than waiting for the next cycle.

6 Bowel / nausea

PCV / vincristine

Use the nausea and preventive bowel plan. Severe constipation, abdominal pain, vomiting or inability to pass wind needs urgent advice.

7 Nerves / movement

Vincristine

Report tingling, numbness, weakness, jaw pain or difficulty walking before the next infusion. Symptoms can affect hands, feet or other areas.

8 Fertility / pregnancy

PCV

Discuss fertility preservation and contraception before starting. Ask for written precautions for patients and partners; do not breastfeed during treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Use your oncology unit’s written emergency plan. Fever, shivering or suddenly feeling unwell needs urgent advice even on days without chemotherapy. Call 112 or 999 for a new seizure, sudden weakness or speech difficulty, loss of consciousness, severe breathing difficulty or uncontrolled bleeding. If you have an established seizure rescue plan, follow it and its emergency instructions. Contact the team promptly for a worsening headache, repeated vomiting, new cough or breathlessness, unusual bruising, yellow skin/eyes or symptoms that prevent drinking. Do not restart held chemotherapy without instructions.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital’s temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

My diagnosis, plan and medicines

Complete these pages with your team. Leave unknown or inapplicable items blank. This record supports the consent discussion and your written prescription.

Diagnosis and plan

Exact tumour diagnosis, site and pathology date

Grade; isocitrate dehydrogenase (IDH) / 1p/19q / O6-methylguanine-DNA methyltransferase (MGMT) results if tested or pending

Treatment goal; surgery / radiotherapy / chemotherapy sequence

Current regimen and phase; planned cycles or review points

What matters to me; lead team and chosen support person

Medicines and safety

Current chemotherapy dose and day-specific calendar

Steroid dose / taper / next review, if prescribed

Seizure medicines / rescue plan, if prescribed

Anti-sickness / bowel / infection-prevention plan

Urgent and out-of-hours contacts; medicine-checking help

SACTCheck patient information. Your team confirms your personal plan. The treatment reference collects the clinical and medicine sources.

My scans, symptoms and support

Record what each test is checking and when the result will be discussed. Scan changes need interpretation alongside symptoms and treatment timing.

Test and purpose	Date / booking contact	Result discussion	Agreed next step

Symptoms and support

Usual symptoms and new changes: headache / seizures / strength / speech

Memory / mood / sleep / daily-function changes

Appetite / weight / bowels / nausea / treatment effects

Rehabilitation, practical help and carer needs

My questions and agreed next steps

My urgent contact plan / what my carer should do

SACTCheck patient information. Your team confirms your personal plan. The treatment reference collects the clinical and medicine sources.

Cycle number: _____ Start date: _____ Next review: _____

Record the current phase, every prescribed medicine and treatment day, doses held, safety tests, symptoms, magnetic resonance imaging (MRI) review and the next authorised treatment. A stamp records treatment; it does not authorise the next dose.

PCV day-specific record · lomustine D1, procarbazine D8–21, vincristine D8 and D29 · copy this sheet for another cycle of the same phase.

Lomustine · Oral chemotherapy capsule

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Procarbazine · Oral chemotherapy capsule

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Vincristine · Chemotherapy given into a vein

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Constipation	□ Usual bowel pattern	□ Mild change: follow the agreed bowel plan	□ Persistent constipation or discomfort: contact the team	□ Pain, vomiting or no stool/wind: urgent assessment	□ Collapse or severe sudden illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Cycle number: _____ Start date: _____ Next review: _____

Record the current phase, every prescribed medicine and treatment day, doses held, safety tests, symptoms, MRI review and the next authorised treatment. A stamp records treatment; it does not authorise the next dose.

PCV next cycle · copy for further cycles · copy this sheet for another cycle of the same phase.

Lomustine · Oral chemotherapy capsule

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Procarbazine · Oral chemotherapy capsule

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Vincristine · Chemotherapy given into a vein

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help
Constipation	□ Usual bowel pattern	□ Mild change: follow the agreed bowel plan	□ Persistent constipation or discomfort: contact the team	□ Pain, vomiting or no stool/wind: urgent assessment	□ Collapse or severe sudden illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

NCCP 00658: adjuvant treatment of grade II glioma after radiotherapy. · NCCP 00658 v2 · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

PCV · 56 days

56 days: lomustine once D1; procarbazine D8–21; vincristine IV D8 + D29.

Phase: _____ Planned cycles: 6

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials
C4	C5	C6
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____
Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.38 · NCCP 00658 v2 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00658 v2 · current regimen**
   https://healthservice.hse.ie/documents/6996/658_V2_PCV-8wk.pdf
2. **Lomustine medac current product information · January 2026**
   https://www.medac.eu/fileadmin/user_upload/medac-eu/SPCs/common_SPCs/Lomustine_medac-spc-common.pdf
3. **Radiotherapy followed by PCV in selected grade II glioma**
   https://pubmed.ncbi.nlm.nih.gov/27050206/
4. **NCI: adult central nervous system tumours · diagnosis and grade education**
   https://www.cancer.gov/types/brain/patient/adult-brain-treatment-pdq
5. **NHS: dexamethasone · use and stopping safely**
   https://www.nhs.uk/medicines/dexamethasone-tablets-and-liquid/how-and-when-to-take-dexamethasone-tablets-and-liquid/
6. **NCI: adult central nervous system tumours — clinical evidence**
   https://www.cancer.gov/types/brain/hp/adult-brain-treatment-pdq
7. **EANO guideline: diffuse gliomas of adulthood, 2021**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7904519/
8. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
9. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
10. **NCI: palliative care during cancer treatment**
    https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
11. **RANO 2.0: response assessment in adult glioma, 2023**
    https://pmc.ncbi.nlm.nih.gov/articles/PMC10860967/
12. **NCCP oral-systemic anticancer therapy (SACT) antiemetics V6: Table 5 p8; timing p4; interactions p11**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_SACT_Induced_Nausea__pWgDCrm.pdf